

For Yujie — what the MRI found, what it means, and what happens next

(Updated 19 Aug 2026, after the MRI on 18 Aug and the radiologist's report. This replaces the 17 Aug version — that one was "what we suspect"; this one is "what we now know". The 17 Aug version was right.)

The one-sentence version

The scan found the thing we were looking for: **the bottom of your spinal cord is being held down ("tethered") by a large lump of fat — a lipoma — that has been there since before you were born.** That one thing explains the constipation and the leaking since childhood, the reduced sensation, the left leg being colder and smaller, the left foot with the high arch and curled toes, the mark on your lower back, and this past year's pain from your bottom down to your heel. It has a name — **tethered cord syndrome from a spinal lipoma** — and it has a treatment.

Exactly what the report says, in plain words

- Your spinal cord ends at the **L4** vertebra. In most people it ends higher, at L1–L2. It's lower because it's held down.
- What's holding it is a **lipoma about 12 cm long** (121 × 49 × 20 mm), attached to the end of the cord and running down inside the sacrum. It is fat — **not a tumour, not cancer**. The doctors gave you contrast dye during the scan specifically to check that, and it confirmed fat.
- The lipoma has grown into the space where the **left L5 and S1 nerves** leave the spine and is pressing on them. Those two nerves run down the back of the leg to the heel. **That is your leg pain.**
- The mark on your lower back is **not a birthmark**. It is a tiny tunnel in the skin — a **dermal sinus** — that formed at the same time as the lipoma. Whether it connects all the way inside is not yet certain (the neurosurgeon will check). Until then: **don't squeeze it, don't press on it, don't put anything on it**, and if it goes red, swells or leaks — or you get a fever with a stiff neck or bad headache — go to Emergency that day.
- Your discs are fine. This was never a disc problem.
- Your bladder test from last October fits too: a small, overactive bladder that doesn't empty properly (300 ml left behind) is exactly what a tethered cord does. It wasn't "just an overactive bladder", and it's fine that you never took the tablets.

Why you have this — and what it is *not*

This formed in the third to fourth week after conception — before your mum knew she was pregnant. At the very bottom of your back, the layer that becomes skin and the layer that becomes the spinal cord didn't separate quite cleanly, and a little tissue that got in the wrong place became fat stuck to the cord. That's the whole cause. **Nothing you did, nothing your parents did, nothing in the pregnancy.** It happens to a small number of babies everywhere and no one knows why. It's usually found in people's twenties or thirties, when the pain starts — like you. It is in the same family as spina bifida, but the closed, hidden kind: your skin is intact, your brain is not involved, and it doesn't get worse suddenly.

What happens next

1. **Neurosurgeon.** Dr Guan has already referred you to **Dr Jeffrey Brennan**, a spinal neurosurgeon in Newtown who operates at RPA. We're booking the appointment now. Take: the MRI report, the bladder test report, photos of your feet and the mark on your back, and the one-page summary Shane is writing.
2. **The likely treatment is an operation to release the cord** — through the lower back, the surgeon separates the fat from the cord and cuts the tether, and removes the skin tunnel. It's a well-established operation, planned not emergency, a few days in hospital, some weeks of recovery.
3. **What the operation is for — honestly:**
 - **Pain:** most people improve clearly.
 - **Stopping it getting worse:** this is the main reason to do it — the pressure on the leg nerves and the recent changes in your bowel are signs it is progressing.
 - **Bowel, bladder, sensation:** some improve, most stabilise, but what has been abnormal for 29 years often doesn't fully come back. So the pelvic-floor and bowel work still matters — it will just finally be aimed at the right thing.
 - **The foot** stays as it is.
1. **Children:** this does **not** affect your ability to have children. When the time comes, your pregnancy needs a bit more planning (the anaesthetist must know about your back — no standard epidural without a specialist plan; high-dose folic acid before you conceive; a bladder plan). Women with this have healthy pregnancies and babies. Better to have the operation first, then plan.

Go to Emergency the same day if

- new numbness in the perineum, buttocks or inner thighs
- suddenly can't pass urine, or it becomes very hard to start
- sudden or clearly worsening weakness in a leg or foot
- losing control of your bowels without feeling it
- **fever with a stiff neck or severe headache, or the mark on your back becomes red, swollen or leaks**

Lastly

Nearly thirty years of "that's just how I am" turned out to be one thing, with a name, that a surgeon knows how to treat. It's not your fault and it was never in your head. Now it's a plan.